How to Say It

By Beth Stillman

SPEAKER: **(Pg 77-80)** “I’m sorry. He's dead. We did everything we could."

Is he breathing? Is he bleeding? Is his heart beating?

“I’m sorry. He's dead. We did everything we could."

He’s dead . . . he’s dead . . . he’s dead.

INTRODUCTION

Every patient death is a different situation; but for health care professionals, one thing is for certain: Telling someone their loved one has died is never easy.

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SPEAKER: I know what happens after you die. I take your family into a quiet room, with Kleenex, and then I say the word "dead." Not "expired" (because you are a person, not milk), and not "passed on," because families always want to believe that means I just transferred you to another hospital.

Dead.

I have to say it.

That's basically all they taught us about how to deliver bad news in medical school. A one-hour lecture.

So we learned by watching our teaching physicians. We were their constant companions in this sort of theater of the bereaved, lurking in doorways and at bedsides and the hospital's ER, waiting to see how soft they made their voices. When did they touch someone on the shoulder? How much medical jargon did they use before getting to the word "dead"?

When you train to become a doctor, they don't really teach you about death. They teach you how to prevent it, how to *fight* it, how to ***say*** it, but not how to *face* it.

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So on one of my first nights as a teaching physician in the emergency room, as we work on the body of a sixteen-year-old boy with eight bullet holes in his chest and abdomen, we are almost angry at his body for not responding to our efforts.

Is he breathing? Is he bleeding? Is his heart beating?

I go to the head of the bed, and I hook him up to a respirator that breathes for him.

We put tubes ... everywhere. A large-bore IV goes into each arm, an even larger one into his groin, and through that we start pressure-bagging type-0-negative blood, trying to replace what he's lost. I call for another unit of blood, but no matter how fast we work, we can't work fast enough.

The monitor begins to sound this shrill insect whine meant to alert us that the patient is crashing, which we already know, so it feels less like a warning and more like a rebuke.

Then we lose his blood pressure and his pulse. But he's sixteen.

So I perform a trauma Hail Mary. I grab a ten-blade scalpel and make a deep incision from the nipple all the way down to the bed. I take the scissors and cut through the intercostal muscles. I take the rib spreaders, push them between the ribs, and crank his chest open.

There's a giant gush of blood and then a moment of stillness, like the second after a lightning strike. Even his blood smells metallic, like ozone.

I reach my hands into his chest, and I put them around his still heart. I begin pumping it for him, feeling for damage. I slide my fingers down the length of his aorta, but it is so riddled with holes that the frayed pieces disintegrate in my hands.

The first time I had to be the one to break that news to a family, I was in my second year of residency. I remember I had to do it in the patient's room, because his adult daughter refused to leave his bedside.

I said, “I’m sorry. He's dead. We did everything we could."

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And then I was supposed to step out of the room, give her a few moments alone. But I was paralyzed, rooted to the spot by a sense of failure and loss. When I looked at the bed, I couldn't stop imagining my own father lying in it.

My attending physician must have seen what was going on in my face, because she grabbed me by the arm and she dragged me outside.

She said, "Don't you ever do that again. Don't you *EVER* pretend that grief belongs to you when it doesn't. One day the person you love will be in that bed. But today you say you're sorry, you mean it. Then you have to walk away."

I look up from the sixteen-year-old boy and see that my own audience has formed. They wait to see what I will do next.

I realize that in front of me is a gaping hole, and the boy's family will probably be here very soon, so I turn to the surgery resident, and I say, "Listen, you have to get this kid closed up as fast as you can."

Not ten minutes go by when we hear the sound of a woman demanding to be let in.

We are not ready.

We are shoving tubes and gauze and surgical supplies into giant trash bags. Security is trying to keep her out, but she is a tsunamic force. We barely had this boy closed up and half covered with a sheet when I see her standing in the doorway.

Clearly his mother. And she goes absolutely quiet. “I’m sorry. He's dead. We did everything we could."

She takes a running leap toward the body. The nurse at the head of the bed sees a large needle still attached to the sutures holding him together and plucks it off the table just before his mother lands on top of his body, trying to protect it with her own.

She starts keening. It's a terrible sound.

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I repeat, “I’m sorry. He's dead. We did everything we could."

She slides off his body. I see her touch the boy's fingers to her mouth briefly before holding them against her cheek. I leave the room as soon as the social worker enters, motioning for everybody to follow me out.

I think, ***That's what they can learn from watching me: how to walk away.*** And without a moment's break, I go to see the next patient. Because there are forty people in the waiting room who all want immediate attention, and they can't know that I still feel the dead boy's heart in my hands like an anchor.

But I know that if I don't put it down now, I might never remember that this loss doesn't belong to me.

One day that grief will be mine. But not tonight.

SOURCE INFORMATION

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AWARD HISTORY

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2019 National Qualifier – MS
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